Akash Institute of Medical Sciences & Research Centre (AIMSRC)

Patient Name:	N/A	**Age/Gender:**	N/A/N/A	**Patient ID:**	N/A
Study Date:	N/A	**Ref. Doctor:**	N/A	**Accession No:**	N/A
Reported Date:	7/3/2026, 3:34:50 pm	**Modality:**	N/A	**Body Part:**	CHEST

CHEST REPORT

History:

The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.

Findings:

The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.

Key Images:

Conclusion:

The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.The chest radiograph (CR Chest) shows that both lung fields are clear with no evidence of focal consolidation, mass lesion, or cavitary changes. The bronchovascular markings appear normal in distribution. There is no pleural effusion or pneumothorax noted. The trachea is central in position. The cardiac silhouette is within normal size and configuration. Both hemidiaphragms are normal in contour and position, and the costophrenic angles are clear. The visualized bony thoracic cage appears intact with no obvious fractures or destructive lesions. Soft tissues are unremarkable. Overall radiographic features are suggestive of no active cardiopulmonary pathology.

Reported By:

Akash

akash
MBBS
Signed on 28/2/2026, 1:14:36 pm

Approved By:

nydile

Nydile D
MBBS, MD (Radiology)
Signed on 28/2/2026, 1:14:40 pm